National Cancer Control Programme (NCCP) 00660 · clinician draft

# Fluorouracil + folinic acid every 14 days

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Fluorouracil treatment day

**Active medicines:** Folinic acid + Fluorouracil

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally once | Before the fluorouracil-only treatment visit<br>NCCP NCIS V8 low-risk default. Review QT risk, interactions, constipation and hepatic function; clinician may select an alternative. |
| Metoclopramide | 10 mg orally up to three times daily as required | Rescue; prescriber confirms duration<br>Check renal function, neurological contraindications and interactions. |

### Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.
- Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00660 v2b · NCCP NCIS antiemetics V8

Sources checked 2026-11-04 · SACTCheck v0.84.32 · Regimen-level drafts; prescriber and oncology-pharmacy verification required